

A stable movement pattern can identify a nerve

Describe the weak movement and pupil before assigning an ocular motor palsy.

Motor patterns

Nerve	Movement clue
III	Ptosis; down-and-out eye; weak adduction/elevation.
IV	Hypertropia and weak depression in adduction.
VI	Esotropia and impaired ipsilateral abduction.

Clinical meaning

An acute pupil-involving third nerve palsy needs urgent angiographic evaluation for PCOM aneurysm. Left fourth nerve paresis causes left hypertropia worse on right gaze and a compensatory right tilt. Multiple deficits with V1/V2 sensory loss suggest cavernous disease. Forced duction helps distinguish mechanical restriction from a paralytic deficit.

Fatigue can imitate several ocular motor patterns

Ocular MG causes fluctuating painless ptosis or diplopia without a pupil defect.

Ocular MG clues

Fatigability

Painless ptosis or diplopia worsens late in the day or on upgaze.

Pupils

Normally spared despite changing ocular motor weakness.

Ice pack

Improvement of ptosis supports the diagnosis.

Clinical meaning

Antibody and neuromuscular assessment support MG; CT thorax checks for thymoma. Pyridostigmine provides symptomatic treatment with neurological follow-up. Ask about swallowing, speech, limb weakness and breathing. Respiratory deterioration raises concern for myasthenic crisis. A fluctuating deficit need not fit one nerve, and normal thoracic imaging does not exclude MG.